M29 Naloxone

Scope of Practice	EMT
Classification	• Opiate Antagonist / Opiate Antidote
Usual Presentation	• 0.4mg/1ml ampule • 4mg pre-filled intranasal spray
Mechanism of Action	• Naloxone competitively binds with opiate receptors reversing the effect of opiate agonists such as morphine.
Indications	• Opiate overdose
Contraindications	• Known allergy
Precautions	• Severe hypoxia prior to administration
Side Effects	• Withdrawal symptoms (including arrhythmias and seizures) • Hypoxia-related agitation after rapid reversal without pre-oxygenation
Adult Dose & Administration	• 0.4mg IV/IO/IM (repeated every 3 minutes as required, maximum 2mg) • 4mg **IN** (repeated every 3 minutes as required)
Pediatric Dose & Administration	• 0.01mg/kg IV/IO/IM (repeated every 3 minutes as required)
Use in Pregnancy and Lactation	• Pregnancy: Safety has not been established. However, naloxone should be administered when clinically indicated. • Lactation: Safety has not been established. However, naloxone should be administered when clinically indicated.
Additional Information	• IV or IN is the preferred method of administration • All cases of opiate overdose should be transferred to hospital even if naloxone has been effective. • If maximum dose has been reached, strongly consider other causes of unconsciousness before further administration. • For intranasal pre-filled spray: • Do not 'test' or 'prime' before use. • Titrate naloxone carefully to reverse respiratory depression only, while monitoring to prevent fully awakening the patient which may cause excessive aggression.